



Guidelines

SCGOPHCG Acute Stroke Guideline: Code Stroke

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

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Introduction

All stroke patients should be managed as a time critical emergency: Rapid assessment and management of stroke is fundamental to reduce mortality and disability.

This guideline details the acute stroke pathway for all patients within SCGOPHCG, and in accordance with the Australian Commission on Safety and Quality in Health Care Acute Stroke Clinical Care Standard 2019, aims to improve the early assessment and management of patients with acute and subacute stroke, to increase their chance of survival, maximise their recovery and reduce their risk of another stroke.

SCGOPHCG Operates in accordance with:

- Australian Commission on Safety and Quality in Health Care Clinical Care Standard: Acute Stroke
- Government of Western Australia, Department of Health. Protocol for Intravenous Thrombolysis in Acute Ischaemic Stroke: 2018.
- Government of Western Australia, Department of Health. Endovascular Clot Retrieval Service Plan for Acute Ischaemic Stroke. 2016.
- Government of Western Australia, Department of Health. The Model of Stroke Care for Western Australia. Neurosciences Health Network: 2012.
- Stroke Foundation Clinical Guidelines for Acute Stroke Management 2017.
- Administration of Alteplase® in Acute Ischaemic Stroke; Standard Operating Procedure #7, Acute Stroke Unit (G51) Sir Charles Gairdner Hospital: 2019.

Risk Statement

Non-compliance with this guideline may:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☐
Other:	☐		

Definitions

Tenecteplase	Tenecteplase: the current medication indicated for the treatment of thrombolysis in acute ischaemic stroke.
Decompressive Hemicraniectomy	Decompressive hemicraniectomy is a surgical technique involving removal of bone tissue (skull) and incision of the dura mater covering the brain to relieve the increased intracranial pressure, allowing swollen brain tissue to herniate upwards through the surgical defect rather than downwards to compress the brainstem.
Endovascular Clot Retrieval (ECR) Pathway	ECR Pathway is where patients are transferred to SCGH for hyper- acute stroke care i.e., mechanical thrombectomy or thrombolysis. Once this treatment phase is complete and the patient is deemed medically stable these patients are to be repatriated back to their home catchment hospital within 24 hours (appendix 1 and 2).
Large Vessel Occlusion (LVO)	Large Vessel Occlusion refers to a blockage in one of the large cerebral arteries (MCA/ACA/PCA) that could be amenable to mechanical thrombectomy.
Malignant MCA Stroke	Malignant MCA is when a middle cerebral artery infarction creates space occupying cerebral oedema resulting in neurological deterioration and potentially irreversible disability.
Mechanical Thrombectomy (MT)	Mechanical Thrombectomy (MT): a minimally invasive procedure performed via angiography, in which a catheter passes up into the brain to remove the clot in the blocked blood vessel (also referred to as Endovascular Clot Retrieval [ECR]).
Modified Rankin Score (mRS)	mRS is a 6-point scale used to rate a patient's premorbid stroke function. Scores range from 0 to 5 (with 6 being deceased) (appendix 4).

Neurological Imaging and Interventional Service of Western Australia (NIISWA)	NIISWA is the radiology procedural area where mechanical thrombectomy is performed.
National Institutes of Health Stroke Scale (NIHSS)	NIHSS is a scale used in acute stroke assessment to determine the impairment caused by the stroke. It is composed of 11 items which rate specific patient abilities. Scoring is from 0 (normal function) to 4 (completely impaired). The score is calculated by adding the number for each item. The higher the final score, the more impairment (appendix 3).
Rapid Arterial Occlusion Evaluation (RACE)	RACE is a 5-item score used pre-hospital (i.e., St Johns Ambulance) to identify a large vessel occlusion. Patients with a LVO (score ≥ 5) are to be transferred to a ECR hospital immediately for further assessment and treatment (appendix 4).
Stroke	Stroke: Sudden and unexpected damage to brain cells that causes symptoms that last for more than 24 hours in the parts of the body controlled by those cells. Stroke happens when the blood supply to part of the brain is suddenly disrupted, either by blockage of an artery or by bleeding within the brain.
Stroke mimics	Are stroke-like symptoms other than an ischaemic or haemorrhagic cause i.e., seizure, migraine, delirium, or syncope.
Thrombolysis	Is the process of blood clot break down (used interchangeably with Tissue Plasminogen Activator).
Tissue Plasminogen Activator	A drug which acts to dissolve the blood clot that is causing the ischaemic stroke
Wake up stroke	Is where a person has gone to sleep well (had no stroke symptoms) however upon waking has stroke symptoms. Stroke 'onset time' is the 'last seen well' time before the person went to sleep.

Procedure

A Code Stroke (Stroke Call) is initiated to provide an immediate coordinated response (assessment and intervention) by the Stroke Team for an acute stroke.

The priority of the 'Code Stroke' Guideline / Pathway is for a prompt succession of the following:

- 1) Initial assessment and patient preparation
- 2) CT Scan- identify Large Vessel Occlusion (LVO) or infarct for potential mechanical thrombectomy or thrombolysis
- 3) Intervention of Mechanical Thrombectomy (MT) in the Neurological Imaging and Interventional Service of Western Australia (NIISWA) or administration of thrombolysis (tPA)
- 4) Facilitate post interventional acute stroke care

1.0 SCGOPHCG Code Stroke Activation

The SCGOPHCG Code Stroke pathway ([Appendix 1](#)) is divided into 'External and Internal Pathway':

- **External** – Activation by the ED Consultant after SJA pre-hospital

acute stroke notification or for a patient suspected with an acute stroke within the ED. All OPH patients will be considered as part of this external pathway.

- **Internal** – Activation by any staff member on reviewing a patient with a suspected acute stroke.

Switchboard will issue a page stating Stroke Call and location. ED Stroke Calls will have an estimated time of arrival (ETA).

**For Acute Stroke Inter-Hospital Referral please see [appendix 2](#)*

SCGOPHCG Code Stroke '55' Criteria

- Clinical Diagnosis of stroke (Impairment of language, motor function, balance, cognition and/or gaze, vision, neglect).

AND

- RACE ≥ 2 and time of onset ≤ 12 hours (including wake up stroke)

OR

- RACE > 4 and time of onset between 12 - 24h

OR

- Pre-notification from SJA patients with RACE* ≥ 5 and onset ≤ 12 h

* For RACE score see [appendix 4](#)

1.1 SCGH Code Stroke Activation:

- A Code Stroke (Stroke Call) is activated by dialing '55' and stating, 'Code Stroke' or 'Stroke Call', the location and the staff member's name.
- ED activated stroke calls will provide an ETA of ambulance to SCGH.

1.2 OPH Code Stroke Activation: (via Code Blue at OPH)

- All acute strokes are reviewed by the Resident Medical Officer (RMO) immediately if patient is unstable a Code Blue must be activated
- During business hours RMO will liaise with the treating Consultant and Ward CNM. After hours RMO will liaise with on call Consultant and After Hours CNM.
- Ward Shift Co-ordinator/After Hours CNM will contact SJA for transfer to SCGH
- St John Ambulance (SJA) transfers patient to SCGH ED.
- All OPH patients will be considered as part of the SCGOPHCG External Pathway.

2.0 SCGH Stroke Team Response

On receipt of the Stroke Call page:

2.1 For External/ ED Activated Stroke Calls:

- G51 Shift Co-ordinator will immediately organise transport of the Code Stroke backpack to the identified acute stroke location. The G51 Shift Co-ordinator is not part of the Stroke Team.
- ED Nurse must ensure the Code Stroke backpack is available to the assigned resus bay.
- Stroke Team will present to ED ready for the ETA presented by page.

EXTERNAL / ED stroke team

In-hours

- Stroke Consultant
- Neurology Registrar
- Stroke Clinical Nurse Consultant*
- Diagnostic Neuro-radiologist*
- NIISWA Nurse

* After hours there is no Stroke CNC and the AH Radiologist will attend CT for imaging review.

2.2 For Internal/ Ward Activated Stroke Calls:

- G51 Shift Co-ordinator will immediately organise transport of the Code Stroke backpack to the identified acute stroke location. The G51 Shift Co-ordinator is not part of the Stroke Team.
- Stroke Team will present to area located by page.

INTERNAL stroke team

In-hours

- Neurology Registrar
- Stroke Clinical Nurse Consultant*
- Diagnostic Neuro-radiologist*
- NIISWA Nurse*
- HSA

*After hours there is no Stroke CNC and the AH Radiologist will attend CT for imaging review. NIISWA nurse is on call after hours and not present in department.

3.0 SCGH Initial assessment and patient preparation**3.1 For External/ED Activated Stroke Calls:**

- Stroke Team Handover should be succinct (5 minutes) and CT must not be delayed
- For SJA pre-hospital activations - SJA staff will escort patient to the allocated resus bay. The patient will remain on the SJA stretcher until taken to Radiology for Imaging and then transferred to the CT table. Once the patient has had their CT the patient will be transferred to a SCGH ED Trolley.

EXTERNAL / ED Team Responsibilities

SJA Verbal Handover in Resus

- Presenting symptom/ onset time / last seen well time
- RACE ([appendix 4](#))
- Was the incident witnessed & is the witness available
- Past medical history/ medications i.e. anticoagulants
- Observations and blood glucose level (BGL)

Neurology Registrar

- Clarifies history
- Performs neurological assessment and documents National Institutes Health Stroke Scale (NIHSS) in Stroke assessment pro forma
- Confirms premorbid mRS ([appendix 3](#))
- Considers/excludes stroke mimics
- Considers inclusion/exclusion for mechanical thrombectomy and thrombolysis
- Determine (and notify NIISWA anaesthetics directly):
 - Fasting status
 - Drug allergies
 - Any history of prior anaesthetic difficulties

ED Physician

- Ensures IVC, bloods: coagulation profile, FBC, U&E, [VBG]
- Ensures radiology request for 'Stroke Protocol' is completed: CT/Perfusion/Angiogram Scan
- Considers/excludes stroke mimics
- Perform trauma exam i.e. fallen pt.
- Ensures patient is medically stable for transfer to CT

ED Nurse

- Observations including GCS,
- BGL
- Ensure IVC inserted and bloods taken pre CT
- Identify if family/witness are available for history and consent
- ECG - rhythm and rate (time permitting- not priority prior to CT)

Pt must be transferred as soon as possible for imaging in CT

3.2 For Internal/ Ward Activated Stroke Calls:

INTERNAL Team Responsibilities

Neurology Registrar

- Clarifies history
- Performs NIHSS
- Ensures patient is medically stable for transfer to CT
- Review: BP, BGL, bloods: coagulation profile, FBC, U&E, [VBG]

In-hours Team RMO

- Ensures IVC
- Ensures radiology request for 'Stroke Protocol' is completed: CT/Perfusion/Angiogram Scan

Ward Nurse

- Observations including GCS
- BGL
- Ensure IVC inserted and bloods taken pre-CT
- Identify if family/witness are available for history and consent
- Must accompany the patient to CT and then NIISWA for handover if the patient is for mechanical thrombectomy.
- ECG - rhythm and rate (must not delay CT)

After Hours:

- The Neurology Registrar will need to ensure IVC is patent and that a radiology request for 'Stroke Protocol' is completed: CT/Perfusion/Angiogram Scan
- The Neurology Registrar should activate a Code Blue if they require immediate assistance or patient clinical status deteriorates.

Pt must be transferred as soon as possible for imaging in CT

4.0 Imaging / CT

- Stroke Protocol standard Imaging includes non-contrast CT, CT Angiogram and CT Perfusion.
- The patient must be transported to CT scan as soon as possible by the Stroke Team, ED Physician and ED Nurse.
- The patient will remain on SJA stretcher until transferred to the CT table.
- Once the scan is completed the patient must wait in the CT holding bay until the decision whether to proceed to mechanical thrombectomy is decided. If there is to be an increased delay time to decision making, then the patient will return to ED in preparation for mechanical thrombectomy.
- Scans are to be reviewed by the Diagnostic Neuro-radiologist and Neurology Registrar in view for mechanical thrombectomy or thrombolysis
- The Neurology Registrar must consult the on-call Stroke Neurologist for progression to mechanical thrombectomy or thrombolysis.
- If the patient is for mechanical thrombectomy then the patient is to be transported directly to NIISWA by the Stroke Team, ED Physician and the ED nurse.
- If the patient is not for mechanical thrombectomy the patient is to return to ED.

5.0 Intervention

Thrombolysis (Intravenous Tissue Plasminogen Activator administration):

*The decision to proceed to thrombolysis (intravenous Tissue Plasminogen Activator administration) is made by the Neurology Stroke Consultant.

- Thrombolysis must be administered immediately (as soon as possible) once the Neurology Stroke Consultant has approved its use and consent is obtained.
- For dosing and administration refer to the WA Department of Health Protocol for Intravenous Thrombolysis in Acute Ischaemic Stroke – 2018 (located in Code Stroke backpack).
- The patient's ED nurse must be available to assist the Neurology Registrar in the preparation and administration of Tissue Plasminogen Activator.
- Nursing care post thrombolysis:
 - 1:1 Nurse Special is required for the first 8 hours (minimum) for all patients who have received thrombolysis.
 - Care provided as per Tenecteplase and Alteplase - Thrombolysis in Acute Ischaemic Stroke (SCGOPHCG guideline) in conjunction with Alteplase Administration – Nursing Protocol (WA Department of Health Protocol for Intravenous Thrombolysis in Acute Ischaemic Stroke – 2018).
- All patients who have received thrombolysis must be admitted to the Acute Stroke Unit (G51), unless requiring ICU (G44), once a bed is available.
 - In hours: the Stroke CNC will hand over to the G51 Shift Co-ordinator and Bed Manager the need for the Tissue Plasminogen Activator 1:1 Nurse Special.
 - After-hours: the ED Shift Co-ordinator is to liaise with Bed Manager and G51 Shift Co-ordinator the need for the Tissue Plasminogen Activator 1:1 Nurse Special.

Mechanical Thrombectomy (MT):

*The decision to proceed to mechanical thrombectomy is made by the Neurology Stroke Consultant and NIISWA Interventionalist.

*Mechanical thrombectomy and thrombolysis can be used as dual therapy for the treatment of Acute Stroke.

- Patients for mechanical thrombectomy must be transported directly to NIISWA by the Stroke Team and the ED Physician and ED nurse.
- Written Consent must be obtained from the patient or if unable from a family member, ED nurse to assist with NOK phone contact details if required. In-hours this will be performed by the NIISWA Registrar and after-hours by the Neurology Registrar.
- Handover provided to NIISWA.
- If there is any extended delay in NIISWA being ready for the patient, then the patient must return to ED post CT and be prepared for the procedure.
- The Neurology Registrar must notify the Bed Manager that an acute Stroke patient is being transferred to NIISWA and a ICU bed is required for the patient in ICU (G44) post mechanical thrombectomy.
- The Neurology Registrar must provide clinical handover to the ICU (G44) Registrar.
- Only the Stroke Neurology Consultant may deem selected pre-intervention (pre-NIISWA) patients to be admitted to Ward G51. The Neurology Registrar must notify the G51 CNS in hours and the AH CNS after hours to assist G51 with reading to accept the patient for G51 admission. Criteria for these patients is as below:
 - Not for resuscitation
 - Not for re-intervention or escalation of care

For standard ward-based stroke carePotential neurosurgical intervention:

- Neurology Registrar is to consult with Neurosurgery for patients who have had a large hemispheric infarct and are at risk of deterioration (i.e., malignant MCA Stroke) for potential decompressive hemicraniectomy. Acute Stroke patients being nursed on the Hemicraniectomy Protocol for potential deterioration for an emergency decompressive hemicraniectomy can be nursed in the G51: please follow the G51 Hemicraniectomy Standard Operating Procedure. Patients who have had a decompressive hemicraniectomy will need to be nursed within the ICU post-operatively.
- Neurology Registrar is to consult with Neurosurgery for patients who have had a haemorrhagic stroke and are at risk of deterioration for potential neurosurgical intervention i.e., insertion of external ventricular drain (EVD). Stable patients who require close monitoring for neurosurgical intervention can be nursed in the G51. They must have a clear medical plan and consultation with neurosurgery documented.

Patients who are for neurosurgery and require treatments such as EVD will need to be nursed within the ICU or on G52.

6.0 Post interventional acute care

- Patients who have undergone mechanical thrombectomy and have been managed in ICU (G44) may be transferred to G51, post review from the Neurology Stroke Registrar and G51 CNS in hours, or AH CNS after hours, and if assessed as clinically safe.
- Endovascular Clot Retrieval (ECR) Pathway patients can be repatriated once:
 - Neurology Stroke Consultant has reviewed patient and has approved transfer.
 - Neurology Registrar has handed over to the receiving catchment hospital.
 - The patient is medically stable.

Related Documents

[Australian Commission on Safety and Quality in Health Care Acute Stroke Clinical Care Standard](#)

References

Government of Western Australia, Department of Health. Protocol for Intravenous Thrombolysis in Acute Ischaemic Stroke: 2018.

Government of Western Australia, Department of Health. Endovascular Clot Retrieval Service Plan for Acute Ischaemic Stroke. 2016.

Government of Western Australia, Department of Health. The Model of Stroke Care for Western Australia. Neurosciences Health Network: 2012.

<https://www2.health.wa.gov.au/~media/Files/Corporate/general%20documents/Health%20Networks/Neurosciences%20and%20the%20Senses/Model-of-Stroke-Care.pdf>

Stroke Foundation Clinical Guidelines for Acute Stroke Management 2017. <https://informme.org.au/en/Guidelines/Clinical-Guidelines-for-Stroke-Management>

Administration of Alteplase® in Acute Ischaemic Stroke; Standard Operating Procedure, Acute Stroke Unit (G51) Sir Charles Gairdner Hospital: 2019.

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Appendix 1: SCGOPHCG Code Stroke Pathway

EXTERNAL / ED SJA Pre-notification, Self-presenting, patient within the ED		INTERNAL SCGH staff member suspects patient is an acute as per SCGH Code Stroke Criteria	
↓		↓	
CODE STROKE ACTIVATION Dial '55'			
SCGH Code Stroke Criteria - Clinical Diagnosis of stroke (Impairment of language, motor function, balance, cognition and/or gaze, vision, neglect). - All wake up strokes and strokes with onset <12 hours with RACE ≥ 2 - Stroke onset >12 - 24 hours RACE ≥ 4 - Pre notification from SJA patients with RACE* ≥ 5			
↓		↓	
EXTERNAL / ED Team - Stroke Consultant - Neurology Registrar - Stroke Clinical Nurse Consultant - Diagnostic Neuro-radiologist - NIISWA Nurse After hours there is no Stroke CNC and the AH Radiologist will attend CT for imaging review - G51 Shift Co-ordinator to take Stroke Trolley to Code location		INTERNAL Team - Neurology Registrar - Stroke Clinical Nurse Consultant - Diagnostic Neuro-radiologist - NIISWA Nurse After hours there is no Stroke CNC and the AH Radiologist will attend CT for imaging review - G51 Shift Co-ordinator to take Stroke Trolley to Code location	
EXTERNAL / ED Responsibilities		INTERNAL Responsibilities	
SJA Verbal Handover in Resus: - Presenting symptom/ onset time/ last seen well - RACE - Was the incident witnessed & are they available - Past medical history/ medications i.e., anticoagulants - BP and BGL Neurology Registrar: - Clarifies history - Performs NIHSS & confirms premorbid mRS - Considers/excludes stroke mimics ED Physician: - Ensures IVC, bloods: coags, FBC, U&E, [VBG] - Ensures radiology request for 'Stroke Protocol' is completed: CT/Perfusion/ Angiogram Scan - Considers/excludes stroke mimics - Perform trauma exam i.e., fallen pt. - Ensures patient is medically stable for transfer to CT		Neurology Registrar: - Clarifies history - Performs NIHSS - Ensures patient is medically stable for transfer to CT - Review: BP, BGL, bloods: coags, FBC, U&E, [VBG] In-hours Team RMO: - Ensures IVC - Ensures radiology request for 'Stroke Protocol' is completed: CT/Perfusion/Angiogram Scan After Hours: - The Neurology Registrar will need to ensure IVC is patent and that a radiology request for 'Stroke Protocol' is completed: CT/Perfusion/ Angiogram Scan The Neurology Registrar should activate a Code Blue if they require immediate assistance.	
Patient transferred to CT for Imaging			
Scans to be reviewed by NIISWA Radiologist and Neurologist in view for Mechanical Thrombectomy (MT) or Thrombolysis (tPA) <i>* The Neurology Registrar must consult the On-call Stroke Neurologist for progression to MT or tPA.</i>			
↓		↓	
Ischaemic Stroke		Haemorrhagic Stroke	
↓		↓	
LVO / suitable for MT Pt transfer to NIISWA for Thrombectomy +/- tPA If patient suitable for G51 (rather than ICU) this discussion between Neurology Reg/Consultant, NIISWA consultant, and ICU Reg/Consultant to happen prior to treatment Stroke CNC (AH- Neuro Reg) to notify Bed Manager for Acute Stroke Unit Bed		Not for Neuro-surgical Intervention *Neuro Reg to handover to G51 Shift Co-ordinator *Stroke CNC (AH- Neuro Reg) to notify Bed Manager for Acute Stroke Unit Bed	
↓		↓	
All patients who receive GA, sedation or deemed to require 1:1 care by neuro-interventionalist/anaesthetist to be recovered in PACU prior to ICU or G51. Stable patients who don't receive GA or sedation can be transferred direct to ICU/G51		G51 Review potential for Hemicraniectomy Protocol Neuro Reg must consult Neuro Surg immediately If G51 cannot accommodate nurse special Stroke CNC (AH- Neuro Reg) contacts Bed Manager for monitored bed (e.g., ICU)	
ICU For 8 hours for BP monitoring post Thrombectomy	G51 Pre-selected patients Not for resuscitation Not for re-intervention or escalation of care AND For standard ward-based stroke care G51	Depending on intervention and clinical status ICU/G52	
Once Stroke Neurology Consultant has deemed patient is stable and suitable - consider repatriation under ECR Pathway			

Appendix 2: SCGOPHCG Metropolitan Inter-hospital Referral for Acute Stroke

Referring Hospital to contact On-call Stroke Neurologist Via SCGH 6457 3333			
↓		↓	
Ischaemic		Haemorrhagic	
↓		↓	
On-call Stroke Neurologist / Registrar to liaise with NISSWA Interventionist On-call Stroke Neurologist to provide handover to Neurology Registrar		Non-call Stroke Neurologist to liaise with Neurosurgery On-call Stroke Neurologist to provide handover to Neurology Registrar	
Ischaemic Stroke		Haemorrhagic Stroke	
↓		↓	
LVO / suitable for MT Pt transfer to NIISWA forThrombectomy +/- tPA If patient suitable for G51 (rather than ICU) this discussion between Neurology Reg/Consultant, NIISWA consultant, and ICU Reg/Consultant to happen <u>prior</u> to treatment Stroke CNC (AH- Neuro Reg) to notify Bed Manager for Acute Stroke Unit Bed		Non-LVO / not suitable for MT Return to ED and assess for tPA *Neuro Reg to handover to G51 Shift Co-ordinator *Stroke CNC (AH- Neuro Reg) to notify Bed Manager for Acute Stroke Unit Bed	
Not for Neuro-surgical Intervention *Neuro Reg to handover to G51 Shift Co-ordinator *Stroke CNC (AH- Neuro Reg) to notify Bed Manager for Bed		For Neuro-surgical Intervention	
↓		↓	
All patients who receive GA, sedation or deemed to require 1:1 care by neuro-interventionalist/anaesthetist to be recovered in PACU prior to ICU or G51. Stable patients who don't receive GA or sedation can be transferred direct to ICU/G51		G51 If tPA Nurse Special for 8hr Review potential for Hemicraniectomy Protocol Neuro Reg must consult Neuro Surg immediately	
ICU For 8 hours for BP monitoring post Thrombectomy		G51 Pre-selected patients Not for resuscitation Not for re-intervention or escalation of care AND For standard ward-based stroke care G51	
G51 If G51 cannot accommodate nurse special Stroke CNC (AH- Neuro Reg) contacts Bed Manager for monitored Acute Stroke Unit bed (e.g., ICU)		Depending on intervention and clinical status ICU/G52	
Once Stroke Neurology Consultant has deemed patient is stable and suitable - consider repatriation under ECR Pathway			

Note Haemorrhagic Strokes not requiring neurosurgical intervention do not require transfer to SCGH.

Appendix 3: NIHSS

NIHSS			Patient Label		
1. A Level of Consciousness:	Alert Drowsy Stuporous Coma	0 1 2 3	6. A Motor Left leg (if 9 please explain) Hold leg at 30 degrees (always test supine) Drift is scored if leg falls < 5 seconds	Normal Drift Some efforts against gravity No effort against gravity No movement Untestable	0 1 2 3 4 9
1. B Level of Consciousness Questions: How old are you? What month is it?	Answers both correctly Answers one correctly Both incorrect	0 1 2	6. B Motor Right Leg (if 9 please explain)	Normal Drift Some efforts against gravity No effort against gravity No movement Untestable	0 1 2 3 4 9
1. C Level of Consciousness Commands Open and close your eyes Grip and release your hand	Obeys both correctly Obeys one correctly Both incorrect	0 1 2	7. Limb Ataxia	No ataxia Present in one limb Present in two limbs	0 1 2
2. Best Gaze	Normal Partial gaze palsy Forced deviation	0 1 2	8. Sensory	Normal Partial loss Dense loss	0 1 2
3. Visual	No visual field loss Partial hemianopia Complete hemianopia Bilateral hemianopia	0 1 2 3	9. Best Language	No aphasia Mild to moderate aphasia Severe aphasia Mute	0 1 2 3
4. Facial Paralysis	Normal facial movements Minor paralysis Partial paralysis Complete palsy	0 1 2 3	10. Dysarthria (if 9 please explain)	Normal articulation Mild to moderate dysarthria Near unintelligible or worse intubated to other physical barrier	0 1 2 9
5. A Motor Left Arm (if 9 please explain) Extend arms to 90 degrees (if sitting), or 45 degrees if supine (palms down). Drift is scored if arm falls < 10 seconds	Normal Drift Some efforts against gravity No effort against gravity No movement Untestable	0 1 2 3 4 9	11. Neglect (extinction and inattention)	No neglect Partial neglect Complete neglect	0 1 2
5. B Motor Right Arm (if 9 please explain)	Normal Drift Some efforts against gravity No effort against gravity No movement Untestable	0 1 2 3 4 9	Total		

Appendix 4: RACE and mRS

Rapid Arterial Occlusion Evaluation (RACE Score)			
	Item	Deficit	RACE Score
1	Facial Palsy	Absent	0
		Mild	1
		Moderate- Severe	2
2	Arm Motor (left or right)	Normal/Mild	0
		Moderate	1
		Severe	2
3	Leg Motor (left or right)	Normal/Mild	0
		Moderate	1
		Severe	2
4	Head and Gaze Deviation	Absent	0
		Present	1
5.1	Aphasia (if right hemiparesis)	Performs both task	0
		Performs 1 task	1
		Performs neither task	2
5.2	Agnosia (if left hemiparesis)	Recognises arm AND impairment	0
		Does not recognise arm OR impairment	1
		Does recognise either	2
Score		0-9	
RACE Score >5 predictor for LVO			
Triaging Tool in hyper-acute phase for pre-hospital screening to identify large vessel occlusion (LVO) for rapid transfer to intervention stroke centre.			
For use while in transport mode via paramedical service – St John’s Ambulance WA and Royal Flying Doctor Service (RFDS)			

Modified Rankin Score (mRS)	
0	No symptoms at all
1	No significant disability despite symptoms; able to carry out all usual duties and activities
2	Slight disability; unable to carry out all previous activities, but able to look after own affairs without assistance
3	Moderate disability; requiring some help, but able to walk without assistance
4	Moderately severe disability; unable to walk without assistance and unable to attend to own bodily needs without assistance
5	Severe disability; bedridden, incontinent and requiring constant nursing care and attention
6	Dead